

Derbyshire Shared Care Pathology Guidelines

Varicella Zoster Virus (VZV)

Purpose of guideline

This guideline provides information about the definition of significant exposure to VZV and management of exposure in immuno-suppressed patients, neonates and pregnant women.

Definitions

Significant contact with chickenpox is defined as any person who is not immune to varicella-Zoster Virus (VZV) (i.e. not had chickenpox before) who has had:

- Contact with a case of chickenpox from the onset of the rash or 24 hours prior to rash onset, until all the lesions are crusted and there is no further cropping
- Face to face contact
- In the same room as the index case for more than 15 minutes
- Contact with clothing and bedding soiled by discharge from blisters

Significant contact with Shingles is defined as any person who is not immune to varicella-Zoster Virus (VZV) (i.e. not had chickenpox before) who has had:

- Contact with case of disseminated/wide-spread shingles or direct contact with an exposed area of shingles from the day of onset of rash until crusted

Confirmation of chickenpox or shingles in the index case/patient:

- Clinical diagnoses – if unclear ensure a swab is sent
- Microbiological – swab from newly burst blister for VZV PCR. The swab test of choice is a viral swab but if unavailable use any sterile swab placed in a sterile Universal tube (a swab from a charcoal swab pack may be used if it is snapped off in a sterile Universal and the charcoal tube is discarded)

Confirmation of the contacts immune status to VZV:

- Ask if there is a confirmed history of chicken pox
- If there is no stated history of chicken pox or the patient is immunosuppressed check for previous immunity- serum (red top tube) for VZV IgG

Types of prophylaxis

- Oral antivirals (e.g. aciclovir and valaciclovir) are now the first choice of PEP for susceptible immunosuppressed individuals, all susceptible pregnant women at any stage of pregnancy and infants at high risk.

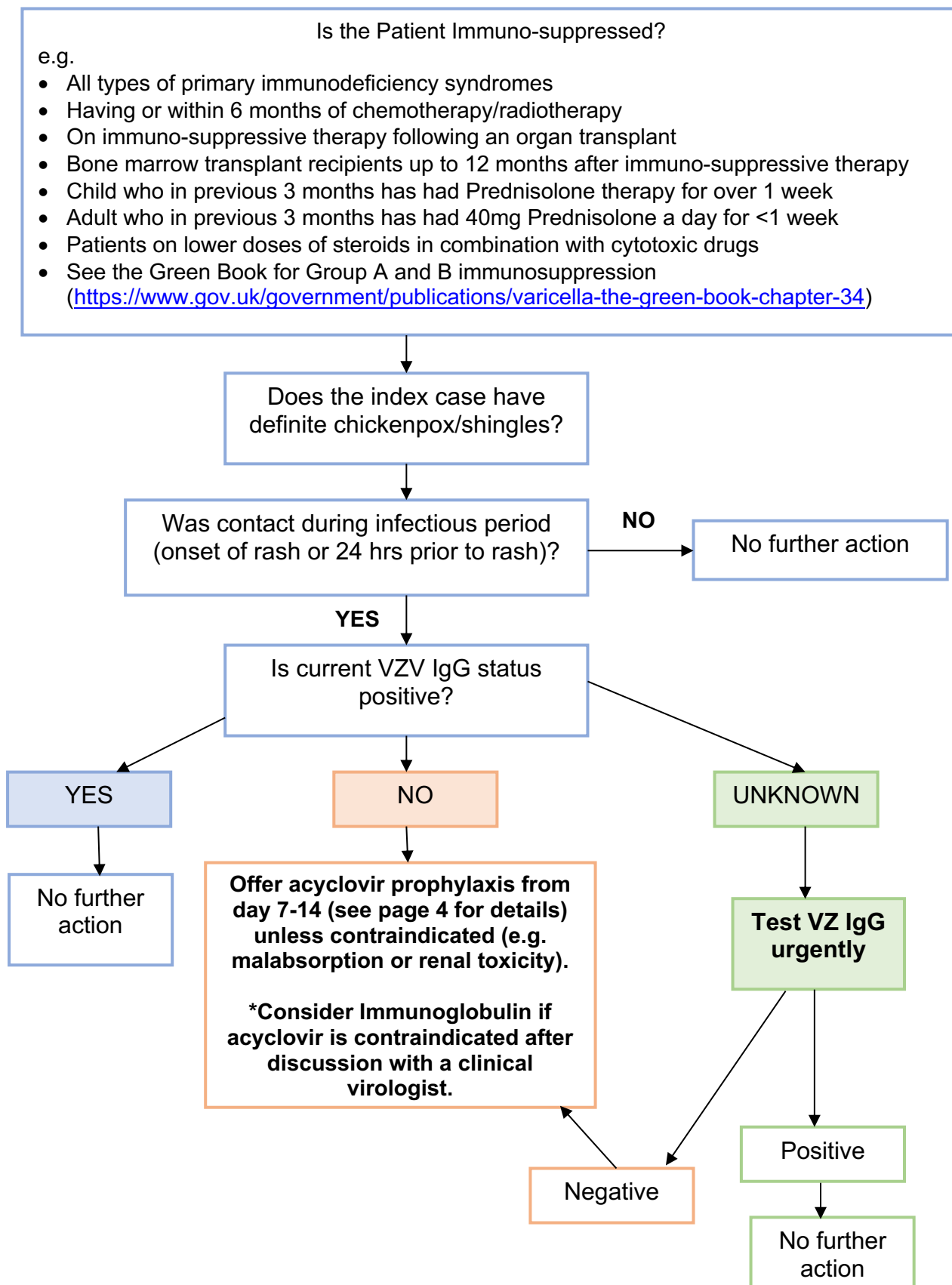
- Varicella Zoster Immunoglobulin (Varitect CP) is administered as a single dose as post-exposure prophylaxis for neonates, exposed to intrauterine VZ infection within the last 7 days of pregnancy, with rash onset in the mother presenting within 1 week of delivery. Varitect CP is unlicensed in the UK but recommended by the UKHSA.
- Normal intravenous immunoglobulin (IVIG) can be used as alternative PEP in patients who cannot receive antivirals, or as an alternative to Varitect CP in neonates

Acyclovir Prophylaxis is recommended in the following settings:

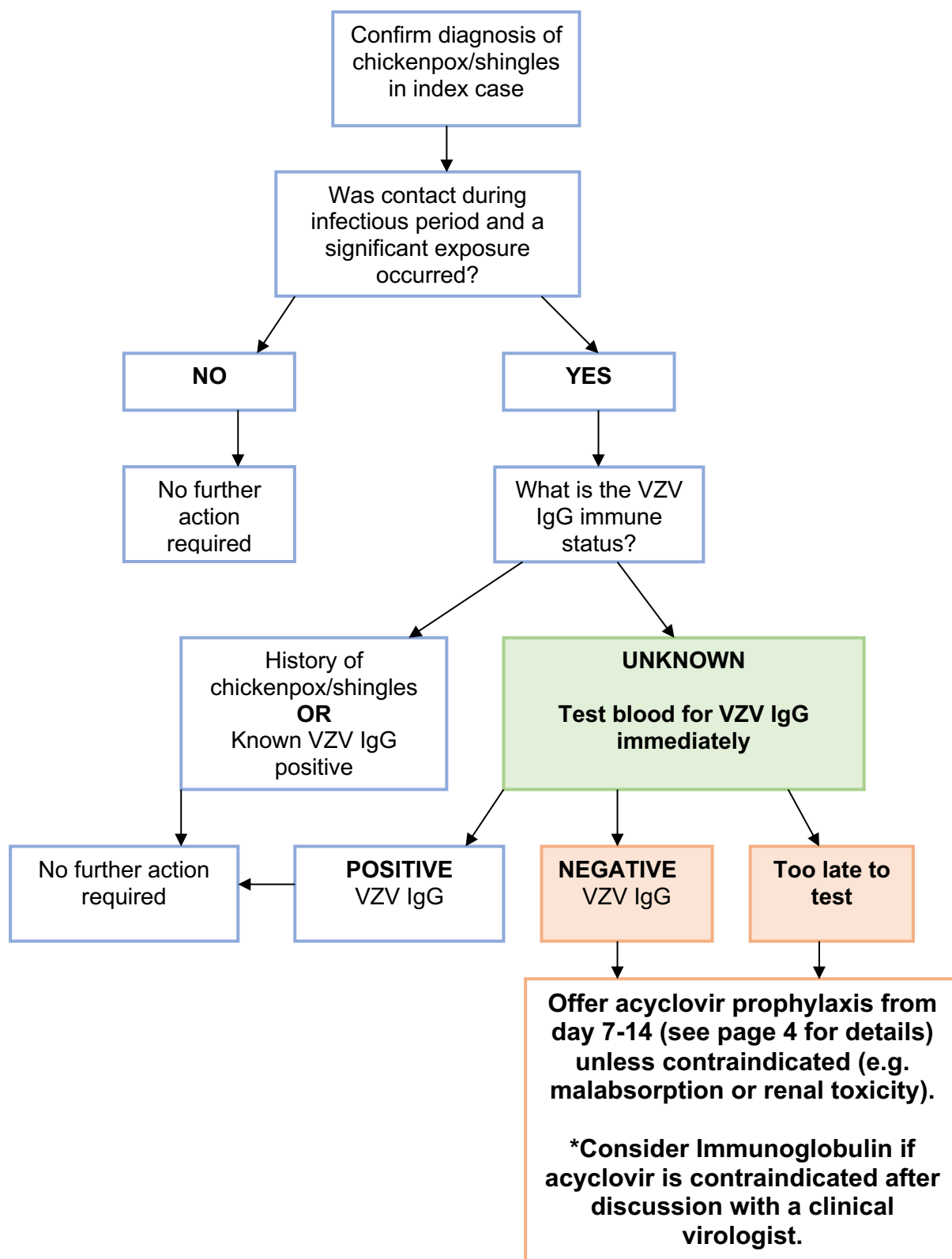
- Pregnant women over are advised to take oral acyclovir at 800mg four times a day or oral valaciclovir 1000mg three times a day from days 7 to 14 after exposure.
- Immunosuppressed individuals are advised the following:
 - Children 4 weeks to 2 years of age – Acyclovir 10mg/kg four times a day, days 7-14 after exposure.
 - Children 2-17 years of age – Acyclovir 10mg/kg (upto a maximum of 800 mg) four times a day, days 7-14 after exposure. Valaciclovir 20mg/kg (upto a maximum of 1000 mg) three times a day, days 7-14 after exposure.
 - Adults - Acyclovir 800 mg four times a day, days 7-14 after exposure. Valaciclovir 1000 mg) three times a day, days 7-14 after exposure.

See following algorithms for assessment and actions required for susceptible immunosuppressed individuals, all susceptible pregnant women at any stage of pregnancy and infants at high risk

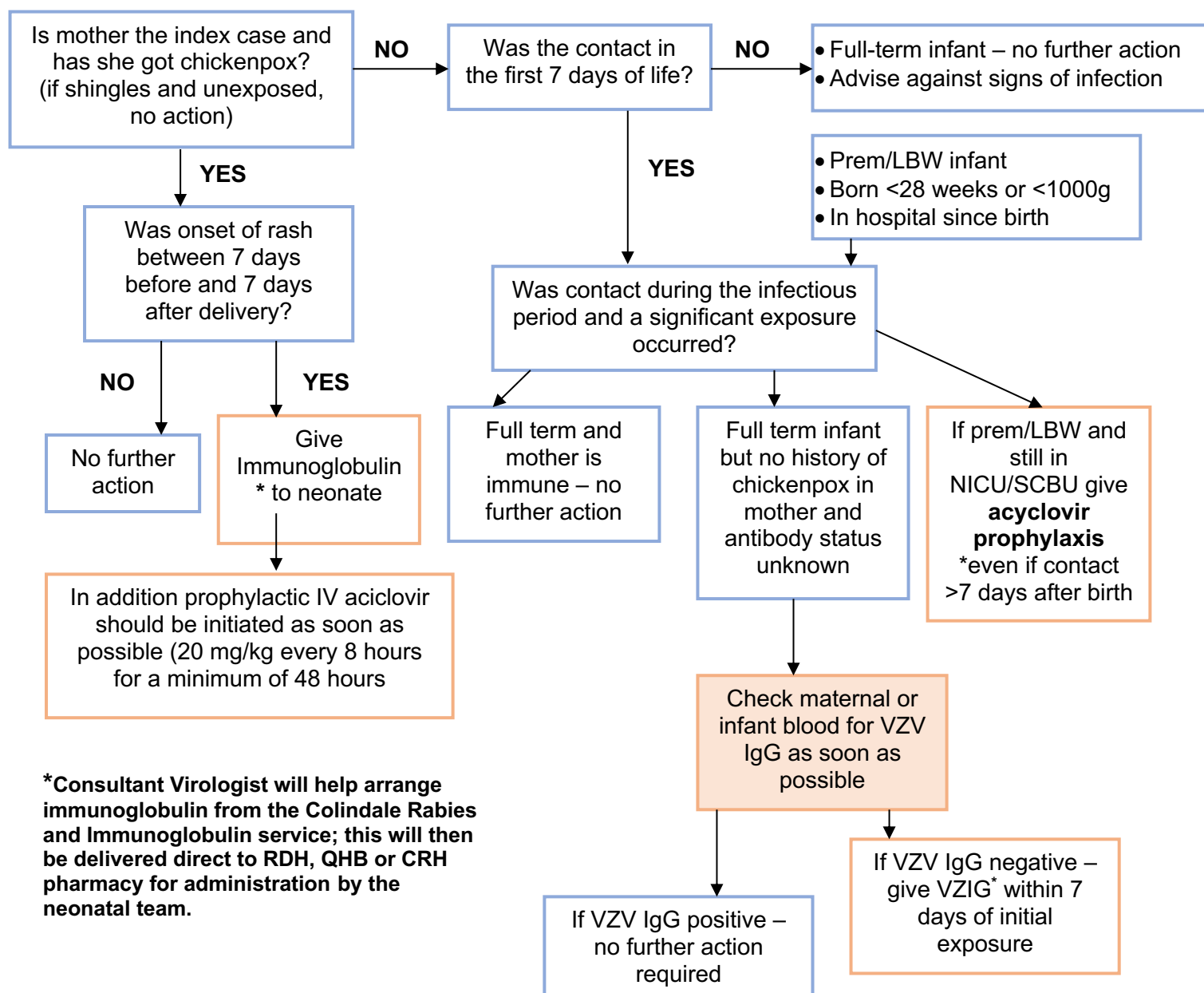
Management of VZV Exposure in immuno-suppressed patients



Management of VZV exposure during pregnancy- see UHDB policy [Chicken Pox in Pregnancy](#) full management advice



Management of VZV exposure in neonates see UHDB full guidance [Chicken Pox in Pregnancy](#)



[Guidelines on post-exposure prophylaxis \(PEP\) for varicella or shingles \(July 2025\) - GOV.UK](#)

Useful Contacts

Derby

Consultant Virologist	10am - 5pm Mon to Fri 01332 788218 (and select option 4)
	Out of hours - please ask for on-call Microbiologist via
	Switchboard 01332 340131
Microbiology Manager	01332 788219

Main Microbiology Lab 01332 788218
Serology Lab 01332 788224

Chesterfield

Consultant Virologist 10am - 5pm Mon to Fri 01332 788218 (and select option 4)
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Microbiology Lab 01246 512270

Burton

Consultant Virologist 10am - 5pm Mon to Fri 01332 788218 (and select option 4)
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Secondary Care UHDB	19/11/2025	Dr C Evans	Consultant Virologist UHDB and CRH
Secondary Care CRH	19/11/2025	Dr C Evans	Consultant Virologist UHDB and CRH
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